

Situation Report of the 17th Ebola Virus Disease Outbreak / DRC

SitRep N°130/MVE-BDBV/09/21/2026

Reporting date: September 21, 2026

Publication date: September 22, 2026

HIGHLIGHTS (last 24 hours)



During the course of September 21, 2026, 40 new confirmed cases of Ebola Virus Disease (EVD) were reported, including 14 community deaths were reported in the provinces of Ituri (19 cases), North Kivu (13 cases), Haut-Uélé (5 cases), and Tshopo (3 cases). In addition, 13 deaths within treatment centers were recorded.

• A total of 384 new samples were analyzed, of which 40 were found to be positive for the virus, representing a positivity rate of 10.4%.

• No new health zones have been affected in the last 24 hours.

ACCUMULATIVE CASE	ACCUMULATIVE DEATH CONFIRMED	LETHALITY	PATIENTS IN ISOLATION/CTE	ACCUMULATIVE HEALED	TRACKING RATES CONTACTS (Of the Day)
7,773	3,759	48.4%	839	1935	84.5%



Affected Provinces and Affected Health Zones



7 Provinces affected

Haut-Uélé, Ituri, North Kivu, South Kivu, Tshopo, Bas-Uélé and South Ubangi



63 out of 167 health zones affected (37.7%)

Summary of key points from the last 24 hours

For the day of September 21, 2026, **40 new confirmed cases** of Ebola Virus Disease (EVD) were reported, including **14 community deaths**.

The confirmed cases are distributed between the provinces of **Ituri (19 cases)**, **North Kivu (13 cases)**, **Haut-Uélé (5 cases)** and **Tshopo (3 cases)**.

Within the CTE, **13 deaths** (11 in Ituri and 2 in North Kivu) were recorded, bringing the total number of confirmed deaths for the day to **27**.

Thirty-three (**33**) patients were declared cured (**17 in North Kivu**, **12 in Ituri** and **4 in Haut Uélé**) after obtaining two negative results for Ebola Virus Disease.

A cumulative total of **7,773 confirmed cases**, including **3,759 deaths**, has been recorded.

Since the start of the epidemic in the Democratic Republic of Congo (DRC), the case fatality rate has been **48.4 %**. The epicenter remains the Ituri province, which accounts for **76.8%** of reported confirmed cases and also represents (**47.5 %**) of new confirmed cases.

In the past 24 hours, no new health zones have been affected. Thus, **63 out of 167 health zones (37.7%)** have been affected in **7 provinces** since the start of the outbreak. Ituri has 28 out of 36 affected health zones, followed by North Kivu (16/34), Haut-Uélé (7/13), Tshopo (7/23), South Kivu (1/34), Bas-Uélé (3/11), and South Ubangi (1/16).

Furthermore, **55 health zones spread across 5 provinces (not including South Ubangi: imported confirmed cases)** remained active while **8 health zones** have not reported any confirmed cases for **42 days** or more.

Table 1. Distribution of confirmed cases and deaths by affected province, as of September 21, 2026.

Province	New confirmed cases (24h)	Confirmed cases	Deaths (confirmed)	Lethality	Affected health zones
Ituri	19	5966	2,737	45.9%	28/36 (77.8%)
North Kivu	13	1438	872	60.6%	16/34 (47.1%)
Haut-Uélé	5	316	129	40.8%	7/13 (53.8%)
Tshopo	3	42	15	35.7%	7/23 (30.4%)
South Kivu	0	3	1	33.3%	1/34 (2.9%)
Bas-Uélé	0	7	4	57.1%	3/11 (27.3%)
South Ubangi	0	1	1	100.0%	1/16 (6.3%)
Total	40	7,773	3,759	48.4%	63/167 (37.7%)

Table 2. Distribution of confirmed cases and deaths by province and zone as of September 21, 2026

Province / Health Zone	Cumulative number			Current situation (24 hours)			
	Case (n)	Deaths (n)	Lethality	New Case	Community deaths (Swab+)	Confirmed deaths within the CTE	Total confirmed deaths
Ituri	5966	2737	45.9%	19	4	11	15
Adja	11	1	9.1%				0
Aru	8	6	75.0%				0
Ariwara	8	2	25.0%				0
Aungba	12	5	41.7%				0
Bamboo	196	41	20.9%	0	0	1	1
Boga	2	2	100.0%				0
Bunia	1666	523	31.4%	4	1	2	3
Damascus	40	9	22.5%				0
Drodro	15	7	46.7%				0
Fataki	81	30	37.0%				0
Gethy	7	3	42.9%				0
Kambala	2	0	0.0%				0
Kilo	34	12	35.3%				0
Komanda	147	91	61.9%	0	0	1	1
Lita	276	138	50.0%	1	0		0
Logo	13	5	38.5%				0
Loiwa	31	8	25.8%				0
Mahagi	4	1	25.0%				0
Mambasa	22	9	40.9%	1	0		0
Mandima	59	33	55.9%				0
Mangala	341	149	43.7%	4	3		3
Mongbwalu	691	322	46.6%	2	0	1	1
Nia-Nia	269	148	55.0%	0	0	3	3
Nizi	761	276	36.3%	5	0		0
Nyankunde	127	36	28.3%				0
Rimba	12	4	33.3%				0

Rwampara	1056	358	33.9%	2	0	3	3
Tchomia	75	25	33.3%				0
To ventilate	N / A	493	N / A				0
North Kivu	1438	872	60.6%	13	8	2	10
Blessed	289	204	70.6%	8	6	1	7
Good	18	7	38.9%				0
Butembo	253	162	64.0%	1	1	0	1
Goma	1	0	0.0%				0
Kalunguta	49	16	32.7%	2	1	0	1
Katwa	544	344	63.2%				0
Kayna	1	0	0.0%				
Kyondo	32	20	62.5%				0
Lubero	20	10	50.0%				0
Mabalako	24	11	45.8%				0
Manguredjipa	12	4	33.3%	1	0	0	0
Masereka	25	9	36.0%				0
Musienene	121	65	53.7%				0
Mutwanga	9	2	22.2%				0
Oicha	14	11	78.6%				0
Vuhovi	26	7	26.9%	1	0	1	1
Haut-Uélé	316	129	40.8%	5	1	0	1
Boma Mangbetu	36	15	41.7%				0
Dungu	1	1	100.0%				0
Gombari	3	1	33.3%				0
Isiro	99	43	43.4%				0
Pawa	70	32	45.7%	2	0	0	0
Rungu	2	1	50.0%				0
Wamba	105	36	34.3%	3	1	0	1
Tshopo	42	15	35.7%	3	1	0	1
Bafwasende	7	1	14.3%				0
Kabondo	5	1	20.0%				0
Lubunga	1	0	0.0%				0
Makiso-Kisangani	22	9	40.9%	2	0	0	0
Mangobo	5	3	60.0%	1	1	0	1
Tshopo	1	0	0.0%				0
Wanie-Rukula	1	1	100.0%				0
South Kivu	3	1	33.3%	0	0	0	0
Miti-Murhesa	3	1	33.3%				0
Bas Uélé	7	4	57.1%	0	0	0	0
Buta	1	1	100.0%				0
Viadana	3	1	33.3%				0
Ganga	3	2	66.7%				0
South Ubangi	1	1	100.0%	0	0	0	0
Bulu	1	1	100.0%				0
Total	7773	3759	48.4%	40	14	13	27

*These are the confirmed deaths that occurred in the various CTEs in the province which are being broken down to be classified by health zone.

NA: Not Applicable.

Table 3. Status of alerts notified by province for September 21, 2026

DPS	Alerts Received		Total alerts	Alerts verified and validated (suspected case)		Alerts verified and invalidated (no suspected cases)		Number of suspects investigated	Number of suspects investigated and transferred to CTE/CT (of the day)
	Living	Deceased		Living	Deceased	Living	Deceased		
Haut-Uélé	7	6	13	6	6	1	0	12	6
Bas-Uélé	67	3	70	4	2	63	1	6	0
Ituri	1066	89	1155	143	86	908	3	225	124
North Kivu	539	18	557	87	18	442	0	105	80
South Ubangi	ND	ND	N / A	ND	ND	ND	ND	ND	ND
Tshopo	103	0	103	7	0	95	0	7	2
Total General	1,782	116	1,898	247	112	1,509	4	355	212

Figure 1. Evolution of cases by date of symptom onset over the last 21 days

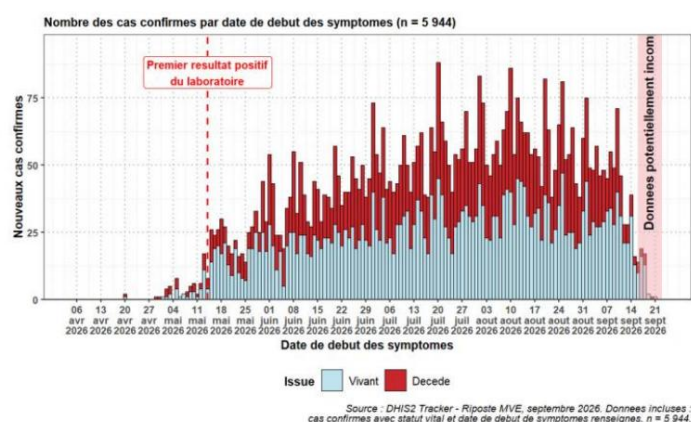
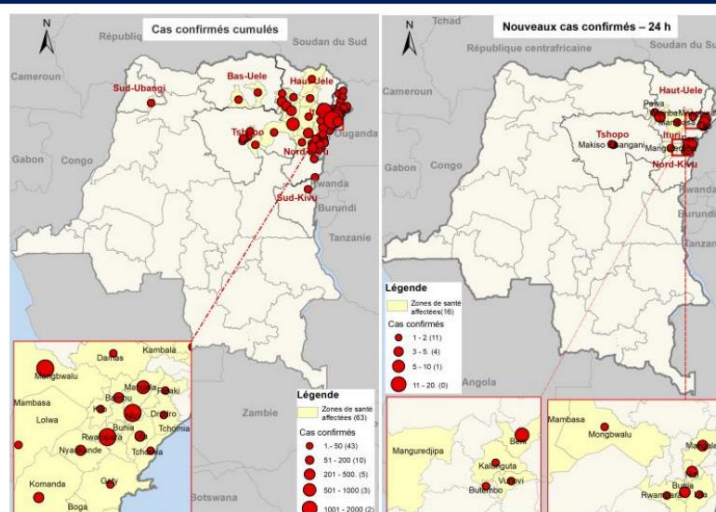


Figure 2. Spatial distribution of confirmed cases (24h and cumulative) by health zone.



1. COORDINATION

1.1. Main Actions

- Supervision of the activities of the pillars of the response and support for the provinces affected.
- Holding coordination meetings with the affected provinces: review of the activities of technical and financial partners in the areas of Beni and Katwa (North Kivu).

2. RESPONSE ACTIONS BY PILLAR

2.1. Epidemiological surveillance

2.1.1. Main Actions

As of September 21, 2026, **1,898 alerts** have been reported; **1,872 (98.6%)** have been verified. Thus, **359 alerts were validated as suspected cases**, **355 (98.9%)** were investigated, and **212** were transferred to CT/CTE (Table 3).

Over the past 24 hours, **26,707 contacts were seen out of 31,612** currently being monitored, representing a follow-up rate of **84.5 %**, higher than the previous day's rate of 81.4%. This rate was 89.6% (189/211) in Bas-Uélé, 85.5% (14,915/17,438) in Ituri, and 84.3% in other provinces.

(280/332) in Tshopo, 83.2% (10,915/13,119) in North Kivu, 81.4% (372/457) in Haut-Uélé, and 65.5% (36/55) in Sud-Ubangi. Consequently, no province ensured follow-up of all contacts by the end of the day.

2.2. Update on checkpoint and point of entry (PoE/PoC) activities

2.2.1. Main actions

Three (3) live alerts were notified to the PoE/PoC in the last 24 hours, including one (1) in Ituri, at the Foner Mambasa PoC (Mambasa Health Zone), and two (2) in North Kivu, at the Kasindi PoE (Mutwanga Health Zone) and the Maboya PoC (Kalunguta Health Zone). All three alerts were validated and investigated within 24 hours (100%; 3/3), and two were transferred to the CT/CTE (66.7%; 2/3), with the suspect in Foner Mambasa refusing transfer.

Table 4. Status of key PoE/PoC indicators by province, as of September 21, 2026

Indicators	Ituri	North Kivu	Tshopo	Haut-Uélé	Overall
Report completeness	96.7% (58/60) 100%	(20/20) 44.4% (8/18) 100% (7/7)			88.6% (93/105)
% of functional strategic PoE/PoC	100%	100%	100%	100%	100%
Number of people who have switched to PoE/PoC	147,003	97,715	11,307	13,493	269,518
% of travelers screened	97.3%	98.2%	97.8%	98.9%	97.7%
% of travelers who washed their hands	97.3%	98.2%	98.7%	98.9%	97.8%
% of travelers made aware	99.4%	98.2%	100.0%	100.0%	99.0%
Number of alerts notified	1	2	0	0	3
Number of validated live alerts (suspected cases)	1	2	0	0	3
Number of bodies recovered today	0	0	0	0	0
% of suspected cases transferred to CTE/CT	0.0% (0/1)	100% (2/2)	N / A	N / A	66.7% (2/3)
% of lifeless bodies swabbed/secured	N / A	N / A	N / A	N / A	N / A
% of suspected cases investigated within 24 hours	100% (1/1)	100% (2/2)	N / A	N / A	100% (3/3)

2.2.2. Challenges

Low completeness of alert entry in DHIS2 Tracker and insufficient mobility in Rwampara, Nizi and Bunia; persistent insecurity hindering the commissioning of the Avakubi PoC (Ituri); In North Kivu, threats against PoC providers on the Butembo axis and vandalism at the Mususa PoC (Musienene Health Zone); in Tshopo, low completeness of reports (44.4%; 8/18).

2.3. Laboratory

2.3.1. Main actions

- **Ituri: 19 new positive results** (15 living and 4 dead) out of 172 new samples received and analyzed (**positivity: 11.0%**).
- **North Kivu: 13 new positive results** (5 living and 8 dead) out of 180 samples analyzed (**positivity of 7.2%**).
- **Haut-Uélé: 5 new positive results** (4 living and 1 death) out of 27 samples received and analyzed at the Isiro laboratory (**positivity: 18.5%**).
- **Tshopo: 3 new cases (2 living and 1 deceased) out of 4 samples** received and analyzed at Isiro laboratory (**positivity: 75%**).

- **Bas-Uélé:** 1 sample received and analyzed in the laboratory, it came back negative.

2.4. Infection Prevention and Control (IPC/EDS)

2.4.1. Main actions

- **In Ituri,** 21 out of 41 rings were opened (51.2%), and 55 out of 55 identified households were decontaminated (100%); 4 out of 4 health facilities were decontaminated, and 45 households received hygiene kits in Mongbwalu. Regarding the health surveillance system (HSS), 98 alerts were recorded (including 4 postponements from the previous day), 76 HSS were carried out (80.9%; 76/94), and 68 bodies were recovered.
- **In North Kivu,** 83 out of 103 infection control rings were opened (80.6%), 36 out of 95 households were decontaminated (37.9%), and 80 out of 97 health facilities were decontaminated (82.5%). Fourteen infection prevention and control (IPC) scorecard assessments were conducted in 14 health establishments with an average score of 32%, and the staff exposure risk assessment classified 6 out of 28 staff members as high risk. Regarding the health monitoring system (HMS), 52 alerts were received (plus 23 postponements from the previous day), 52 bodies were swabbed, and 58 HMS tests were carried out; 17 alerts were postponed to the following day.
- **In Tshopo,** the training of 50 PPLs (Personal Protective Equipment) in the Makiso-Kisangani Health Zone concluded; decontamination of 4 households with incineration of belongings and of the La Borne nursery school; 13 ESS (Emergency Social Services) were monitored and supported, with 139 service providers briefed on standard precautions. Regarding EDS (Emergency Response Scheme), no alerts were received and 1 EDS was carried out (postponed from the previous day's assessment at the CTE - Technical Center).
- **In Bas-Uélé,** 16 out of 17 healthcare providers (94%) and 11 hygiene workers were briefed on IPC-WASH in 5 health facilities in the Ganga health zone; 7 health facilities in the Buta health zone received IPC supplies with support from FHI 360, the Buta treatment center was bio-cleaned and decontaminated, and the Limba health center (Viadana health zone) achieved an IPC score of 67.7%. As for the emergency health services, 2 alerts were received and 2 bodies were swabbed (Buta and Titule health zones).

2.4.2. Challenges

In North Kivu : the health zones of Biena, Masereka, Kyondo, Mabalako, and Vuhovi remain uncovered by decontamination activities due to a lack of training and motivation among the dedicated teams. In Tshopo, there is a shortage of infection prevention and control (IPC) supplies in social and health establishments (SHEs), households, schools, and markets; IPC kit coverage is only partial in SHEs (38.1%; 37/97); and the incinerator at the Makiso-Kisangani General Referral Hospital (HGR) remains out of service.

2.5. Holistic care

2.5.1. Main actions

- **In Ituri,** 92 new admissions were recorded during the day, compared to 102 discharges, including 12 recoveries. Thus, 432 patients are hospitalized out of 1015 beds (occupancy rate of 42.6%); 386 out of 411 patients (93.9%) received hot meals, and children under LNPE/SLM were fully covered at the CTE (14/14), at the nursery (3/3), and in the community (27/27).
- **In North Kivu,** 92 new admissions were recorded, compared to 70 discharges, including 17 recoveries; 338 patients are hospitalized, 274 of whom are in standardized facilities with 308 beds, representing an occupancy rate of 66.2%; 64 patients (18.9%) remain under the care of health establishments outside of treatment centers. The Matanda treatment center (Katwa health zone) has officially opened; all 190 patients (100%) received hot meals.
- **In Haut-Uélé,** 11 new admissions were recorded against 6 discharges (4 recovered). Thus, 50 patients are hospitalized for 138 beds, representing an occupancy rate of 36.2%.
- **In Tshopo,** three new admissions and three discharges were recorded. Eleven (11) patients are in isolation out of 31 beds, representing an occupancy rate of 35.5%; 9 confirmed and 2 suspected cases have

received a hot meal, as did 20 suspects and 42/42 (100%) frontline service providers.

- **In Bas-Uélé**, two new admissions of suspected cases were recorded at the CT in Poko, with no discharges; 8 patients are in isolation in non-standard CTs (1 confirmed in Ganga).

2.5.2. Challenges

Treatment remains partially substandard and capacity is limited: **In Ituri**, Ebola Treatment Centers (ETCs) are saturated with suspected cases in Nizi, Mambasa, Lolwa, Kilo, Mangala, and Mandima. **In North Kivu**, 34.2% of hospitalized patients are treated outside of standardized facilities; no beds are available in Butembo and Katwa. **In Tshopo**, the lack of safe blood and limited technical resources persist. **In Bas-Uélé**, the absence of ETCs and ambulances hinders the isolation of cases.

2.6. Vaccination

2.6.1. Main actions

- **In Tshopo**, 3,774 TPL and PPL were vaccinated out of 3,549 pre-registered (106%: adherence after awareness), i.e. 32% of the target of 11,703, including 1,909 in Makiso-Kisangani; 4,000 doses were distributed to the ZS and 726 thawed doses remain in stock, of which 226 expire on September 24, 2026.

2.6.2. Challenges

Low vaccination uptake among people living with HIV and THI (32% of the target population) in Tshopo (32%) and risk of loss of 226 thawed doses expiring on September 24, 2026; in Bas-Uélé, lack of a micro-plan for vaccinating people living with HIV in the 6 priority health zones and need for additional doses of ERVEBO

2.7. Continuity of Care

- **In Tshopo**, 190 cases received the experimental treatment, including 28 confirmed cases (3 under MBP134 and 26 under Remdesivir: 10 cured, 7 deceased, 1 escaped and 8 currently receiving treatment), and 162 benefited from post-exposure prophylaxis (100 completed, 12 interrupted and 50 ongoing); 40 cases of Buruli ulcer are awaiting treatment due to a lack of clarithromycin.

2.8. Risk Communication and Community Engagement (RCCE)

2.8.1. Main actions

- **In Ituri**, 24,919 home visits reached 80,630 people and 717 educational talks reached 14,137 people; 42 mass awareness campaigns reached 14,673 people, 35 community debates brought together 1,001 people and 68 advocacy sessions reached 266 people; 356 community alerts were notified, including 53 deaths, 311 feedbacks were received, of which 275 were resolved (88.4%), and 20 community incidents, of which 11 were resolved (55.0%), for a completeness of 32.1% (9/28 health zones); two customary chiefs from Komanda and Lolwa pledged to support the response.
- **In North Kivu**, 9,916 home visits reached 28,506 people, 71 educational talks reached 1,304 people and 14 mass communication sessions reached 5,298 people; 139 alerts of suspected cases and deaths were notified by community actors out of 450 expected (30.9%); 13 out of 18 feedbacks were resolved within 48 hours (72.2%) and 1,165 out of 4,750 people were engaged in community actions (24.5%).
- **In Bas-Uélé**, 244 home visits reached 1,410 people, 24 educational talks reached 1,287 people, and 2 mass awareness campaigns reached 2,671 people; 542 school principals were sensitized in Buta by the provincial minister, and 47 members of the provincial youth committee were trained on the early detection of Ebola; 8 community alerts were reported and all addressed

- **In Tshopo**, 173 home visits reached 968 people and 3 educational talks were organized for the benefit of 69 people, for a total of 1,493 people sensitized (657 men and 836 women); 3 interactive shows were hosted; 29 community alerts were notified and 2 feedbacks as well as 2 refusals were resolved (100%).

2.8.2. Challenges

Persistent community resistance: **In Tshopo**, confirmed cases who escaped from the Ebola Treatment Center (ETC) (20th and 26th cases, Mangobo Health Zone) remain at home, and 10 contacts of the 20th case refuse follow-up. **In Ituri**, data reporting is low (completeness: 32.1%; 9/28), there is a lack of numerical data on interventions around cases, and demotivation of stakeholders due to unpaid fees. In **North Kivu**, resistance persists in the Lubero Health Zone (Vunyakondomi Health Area), a suspected case in Butembo refuses to be transferred to the Treatment Center, CREC stakeholders are demotivated due to unpaid fees for over four months, and community alerts are low (30.9%; 139/450). **In Bas-Uélé**, reports from the Health Zones are low, there is a lack of communication credit, and the roads are impassable.

2.9. Mental Health and Psychosocial Support (MHPSS)

2.9.1. Main actions

- **In Ituri**, 25 of the 34 confirmed cases (73.5%) from the previous day and 105 of the 154 confirmed cases under follow-up (68.2%) benefited from SMSPS interventions, as well as 25 of the 86 new suspects (29.1%) and 101 of the 114 suspects under follow-up (88.6%); 386 caregivers and 54 PPLs were supported, 17 of the 24 recovered (70.8%) were followed up in the community and 85 results were announced, including 25 positive results; 11 of the 28 health zones are covered and 10 children are being followed up in daycare.
- **In Tshopo**, 7 confirmed cases under follow-up (100%) and 11 suspected cases, including 4 new ones, received support at the CTE Météo; 52 people received psychoeducation, including 40 caregivers and 12 PPLs, as well as 35 members of the deaf-mute community in Kisangani and 30 teachers in Baego.

2.9.2. Challenges

- **In Ituri**, low coverage of SMSPS interventions (11 of the 28 health zones covered); in North Kivu, low coverage around the latest confirmed cases in 9 health zones and absence of nurseries in 10 affected health zones; in Tshopo, need to train the APS of the health zones of Wanie-Rukula and Bafwasende.

2.10. Logistics

2.10.1. Main actions

- Support for the mobility of response teams in all affected provinces; handover of the CTE and laboratory of the ITAV (Butembo Health Zone) and the Matanda CTE (Katwa Health Zone) and provision of 40 motorcycles to the health zones of Musienene, Butembo, Katwa and Beni in North Kivu, where construction continues of the Musienene General Referral Hospital CTE, and of the Bafwasende CTE in Tshopo and transport of IPC and PEC inputs to the health zones of Ganga, Poko and Viadana in Bas-Uélé with the support of WHO.

2.10.2. Security

2.10.3. Main actions

- Continued security monitoring in the affected provinces; in North Kivu, the PCI and EDS (decontamination, swabs and burials) teams were accompanied in the sub-coordinations of Beni and Katwa (100%) and public order was restored without damage at the BCZ after a mood movement of CREC agents demanding their motivation.

2.11. PSEA (Prevention of Sexual Exploitation and Abuse)

2.11.1. Main actions

- In Ituri, 374 people (171 men and 203 women) were sensitized on PSEA in the community of the health zones of Bunia and Rwampara, with the support of the WHO; the display of messages and the distribution of PSEAH leaflets are continuing.

2.11.2. Challenges

- Still insufficient coverage of PSEA activities, with only 13 of the 36 health zones in Ituri (36.1%) aware of the reporting mechanism; lack of mapping of stakeholders and refusal of some partners to align; insufficient mass awareness materials; lack of computer kits and office supplies for the pillar.

KEY MESSAGES

- Active and persistent community transmission in the provinces of Ituri, North Kivu, Haut-Uélé, Tshopo and Bas-Uélé. However, an overall decrease in the number of confirmed cases (**34.4%** : 61 vs 40) is driven by the province of Ituri, which has seen a decrease of **44.1%** compared to the previous day.
- The performance of epidemiological investigations remains satisfactory, with **98.9%** of suspected cases investigated, although the contact tracing rate fluctuates (**84.5%**) resulting from a decline in completeness in several health zones.
- Strengthening priority interventions (surveillance, care, IPC, logistics and community engagement) remains essential to rapidly interrupt transmission chains: Rapidly increase reception capacities in ETC/CT in North Kivu and strengthen cross-border surveillance with South Sudan.
- Strengthen preparedness and response to Ebola Virus Disease-B in the provinces of the River Corridor and surrounding areas.

TIMELINE OF KEY EVENTS

- From April 24 to May 15, 2026, the epidemic progressed from the detection of the first suspected cases to the biological confirmation of the Ebola Bundibugyo virus, leading to the official declaration of the 17th Ebola Virus Disease epidemic in the Democratic Republic of Congo.
- Between May 17 and 18, 2026, national authorities, with the support of partners, strengthened the response by declaring a public health emergency of national and then continental scope, accompanied by the mobilization of funds.



EBOLA PRESIDENTIAL FORCE TASK 17
Ministry of Public Health, Hygiene and Social Welfare
National Institute of Public Health
Public Health Emergency Operations Center
MVE17 Incident Management System

Director General of INSP

Dr. MWAMBA KAZADI Dieudonné

Email: diedonne.mwamba@insp.cd

Incident Manager

Professor Dr. AHUKA MUNDEKE Steve

Email: steve.ahuka@unikin.ac.cd

Operations Manager

Dr. KOMBE NGWAMA John

Editorial Team

Professor Dr. MULANGU KATULUMBA Félix-Médard

Dr. Jules Katusi Biselenge

Mrs. MAYELE AFUA Marie

Dr. DIUR MANANASHI Floryan

Mr. KAMUINA KEBELA John

Contact :

www.insp.cd

contact@sgiebola17.org

Freephone number: 151. www.sante.gouv.cd